Which treatment this guide covers NCCP 00565a covers first-line treatment of adults with locally advanced or metastatic NSCLC with an activating EGFR mutation. This is daily dacomitinib alone; it is not an adjuvant regimen or a chemotherapy combination.

NCCP 00565 · VERSION 3

Dacomitinib monotherapy

The usual starting dose is 45 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription; some patients start lower because of liver function. Treatment continues while it helps and side effects remain acceptable.

Make treatment fit into daily life

Take at approximately the same time each day with or without food and swallow with water. If you miss a dose or vomit, do not take a replacement or extra dose. Take your next prescribed dose at the usual time the following day.

Before starting

Confirm the mutation, treatment setting, prescription, baseline tests, first review and emergency contact.

Taking the tablets

Take at approximately the same time each day with or without food and swallow with water. If you miss a dose or vomit, do not take a replacement or extra dose. Take your next prescribed dose at the usual time the following day.

Supplies and visits

Arrange the next prescription before tablets run out. Keep your blood-test and clinic appointments. The oral regimen itself does not require an infusion line or pump.

If treatment is held

Ask why, what must recover and when review is due. Only the team authorises restarting or changing dose.

At treatment change

Agree ongoing symptom care, scans, contraception and follow-up.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00565 v3 · official regimen
- EMA Vizimpro product information (updated 17 December 2024)
- NCCP SACT antiemetic guidance V6 (2025), oral tables
- NCCP NCIS antiemetic medicines V8 (2025); parenteral scope
- ARCHER 1050 primary PFS: Wu et al., Lancet Oncology 2017
- ARCHER 1050 extended OS: Mok et al., Targeted Oncology 2021

Source reconciled; independent clinical, pharmacy and patient/PPI review pending